

NO PRIOR AUTHORIZATION

Peptide Therapy *Playbook*

FOR PRESCRIBING PROVIDERS

7 Modules · 110+ Pages

DANIELLE ALCALA

NOPRIORAUTHORIZATION.COM

The Peptide Therapy Playbook

KEY PEPTIDES · PATIENT SCREENING · PROTOCOLS · VENDORS · CONSENT FORMS · FAQs

1

Peptide Therapy — Overview & Scope of Practice

What peptides are, who can prescribe them, and why this is the fastest-growing category in wellness

CRITICAL — THE REGULATORY LANDSCAPE IS ACTIVELY SHIFTING

Peptide therapy exists in a complex and rapidly evolving regulatory space. Several peptides previously available through compounding pharmacies have been reclassified or restricted by the FDA in recent years — including BPC-157, TB-500, and others. Before building a peptide program, verify the current FDA and DEA status of every peptide you intend to prescribe. The status of compounded peptides changes frequently. This playbook reflects the general landscape as of 2025 — always verify current status with your compounding pharmacy and legal counsel before prescribing.

What peptides are — and why patients want them

PEPTIDES — THE BASICS

Peptides are short chains of amino acids — the building blocks of proteins — that act as signaling molecules in the body. Unlike hormones that replace a deficient hormone directly, peptides work by signaling the body to increase its own production of hormones, growth factors, or repair proteins. They are used therapeutically to support: growth hormone optimization, fat loss, muscle preservation, injury recovery, immune modulation, cognitive function, sleep quality, skin regeneration, and sexual function. The specificity of peptide signaling — acting on targeted receptors — is what makes them clinically interesting and commercially appealing.

\$48B

Global peptide therapeutics market —
growing rapidly

7,000+

Known naturally occurring peptides in
the human body

\$300–\$600

Average monthly peptide program
revenue per patient

Scope of practice — who can prescribe

PRESCRIBING AUTHORITY FOR PEPTIDES

- MDs, DOs — full prescribing authority in all states

- NPs — independent prescribing authority in full-practice states; collaborative agreement required in others
- PAs — prescribing authority varies by state; collaborative agreement typically required
- Estheticians, RNs without prescriptive authority, medical assistants — cannot prescribe peptides
- Peptides that are compounded require a valid patient-provider relationship and a legitimate clinical indication — not cosmetic use alone

Why peptides belong in an aesthetic practice

1

Your patients are already asking

Aesthetic patients are wellness-forward. They're already reading about peptides on social media and asking their providers about them. Being the practice that can answer intelligently and offer a supervised program builds immediate trust and loyalty.

2

Direct aesthetic benefits

Several peptides — GHK-Cu, BPC-157, GH secretagogues — have documented effects on skin quality, collagen production, healing, and body composition. These outcomes are deeply aligned with what aesthetic patients seek.

3

High-retention recurring revenue

Peptide programs are monthly. Patients on GH secretagogues or healing peptides commit to 3–6 month cycles minimum. Average monthly revenue \$300–\$600 per patient. High lifetime value, low churn when results are good.

4

Natural complement to hormones and GLP-1

Patients already on your hormone or GLP-1 program are ideal peptide candidates. The concerns overlap: body composition, energy, recovery, skin quality. One patient. Multiple revenue streams.

2

Patient Screening & Candidate Selection

Who is right for peptide therapy — and how to open the conversation

OPENING CONSULTATION SCRIPT — PEPTIDE INQUIRY

"Tell me what's driving your interest in peptides — what are you hoping to feel or achieve that you're not experiencing right now?"

Peptide patients often arrive with significant self-research. Meet them where they are. Understand their goals before recommending anything. This question surfaces the right peptide category — body composition, recovery, skin, sleep, or cognitive function — without leading them.

Ideal peptide therapy candidates — by goal

BODY COMPOSITION AND ANTI-AGING

- Adults 35+ experiencing gradual muscle loss, increasing body fat despite stable lifestyle

- Athletes and active patients seeking improved recovery and lean mass
- Patients with documented low IGF-1 levels — growth hormone secretagogues most indicated here
- GLP-1 patients looking to preserve muscle mass during weight loss — peptides complement well

RECOVERY AND HEALING

- Patients with chronic musculoskeletal injuries not responding to standard treatment
- Post-surgical patients (with surgical team awareness and approval)
- Athletes with overuse injuries or slow recovery
- Patients with gut dysfunction or inflammatory conditions (BPC-157 historically used — check current FDA status)

SKIN, HAIR, AND AESTHETIC ENHANCEMENT

- Patients seeking collagen support beyond topical skincare
- Post-procedure patients wanting enhanced healing and skin quality
- Hair loss patients — GHK-Cu has shown promise in hair follicle stimulation
- Patients wanting systemic skin benefits alongside their injectable aesthetic treatments

SLEEP, COGNITION, AND STRESS

- Patients with sleep disruption not fully addressed by hormone optimization
- Patients with stress-related fatigue, brain fog, and burnout
- Patients seeking cognitive enhancement and neuroprotection
- Patients interested in mood support alongside conventional care

Baseline evaluation — what to assess before prescribing

RECOMMENDED BASELINE LABS FOR GH SECRETAGOGUE PROGRAMS

- IGF-1 — the primary marker of growth hormone status; guides indication and monitors response
- Fasting glucose and HbA1c — GH secretagogues can affect insulin sensitivity; baseline essential
- Fasting insulin — metabolic baseline
- CBC and CMP — general health baseline
- TSH — thyroid function; overlapping symptoms and GH affects thyroid axis
- Lipid panel — GH optimization can affect lipids; baseline and monitoring

Contraindications to consider

USE WITH CAUTION OR AVOID

- Active malignancy — growth-promoting peptides are generally contraindicated; consult oncology
- Proliferative diabetic retinopathy — GH secretagogues may worsen
- Uncontrolled diabetes — GH affects glucose; optimize diabetes first
- Pregnancy and breastfeeding — no safety data; avoid
- Known hypersensitivity to peptide components
- Active carpal tunnel syndrome — GH-related fluid retention can worsen
- History of hormone-sensitive cancers — individualized risk discussion required

3

Key Peptides — Clinical Overview & Protocols

The most clinically relevant peptides for aesthetic and wellness practices

ALWAYS VERIFY CURRENT FDA STATUS BEFORE PRESCRIBING ANY COMPOUNDED PEPTIDE

The FDA has reclassified several peptides as bulk drug substances that cannot be compounded. Status changes with little warning. Verify with your compounding pharmacy before building a protocol around any specific peptide.

Growth Hormone Secretagogues — body composition and anti-aging

Sermorelin

GHRH analog — stimulates pituitary GH release

PRESCRIPTION

FDA HISTORY

CLINICAL USE AND DOSING

- Stimulates pituitary to release natural GH — pulsatile, physiologic
- Typical dose: 200–500mcg subcutaneously at bedtime
- Cycle: 3–6 months minimum; often continued long-term
- Results: improved sleep quality (often first), then body composition, energy
- Well-tolerated — side effects generally mild (flushing, tingling)

CLINICAL PEARLS

- Works best when combined with adequate sleep — GH releases during deep sleep
- Inject at bedtime on empty stomach for best response
- Monitor IGF-1 at baseline and 3 months
- Do not inject with meals — insulin blunts GH release
- Historically FDA-approved for pediatric GH deficiency — compounded for adults

Ipamorelin + CJC-1295

GHRP + GHRH combination — the most commonly prescribed GH secretagogue stack

PRESCRIPTION

COMPOUNDED

CLINICAL USE AND DOSING

EXPECTED OUTCOMES TIMELINE

- Ipamorelin: selective GHRP — stimulates GH release without significant cortisol or prolactin spike
- CJC-1295 (without DAC): short-acting GHRH analog — synergistic with Ipamorelin
- Combined dose: Ipamorelin 200–300mcg + CJC-1295 100–200mcg subcutaneous
- Timing: at bedtime, 3–5x per week or daily
- Cycle: 3–6 months; reassess IGF-1 before continuing
- Week 1–2: improved sleep depth and quality — most common first result
- Week 3–4: increased energy, mild fat loss beginning
- Month 2–3: body composition changes visible — lean mass, reduced visceral fat
- Month 3–6: skin quality improvement, hair quality, recovery enhancement
- Monitor: IGF-1 at 3 months. Fasting glucose quarterly.

Tesamorelin

FDA-approved GHRH analog — strongest evidence base

FDA APPROVED

PRESCRIPTION

CLINICAL USE

- Only FDA-approved GHRH analog currently available
- FDA-approved for HIV-related lipodystrophy; prescribed off-label for body composition
- Dose: 1–2mg subcutaneous daily
- Strongest clinical evidence for visceral fat reduction of all GH secretagogues
- Available commercially (Egrifta) — expensive without insurance

WHY IT MATTERS

- Most robust evidence base — ideal for evidence-conscious patients
- Documented visceral fat reduction — appeals to metabolic health focus
- Monitor glucose — can reduce insulin sensitivity
- Good alternative when compounded CJC/Ipamorelin availability is limited
- Pairs well with GLP-1 programs for enhanced body composition

Healing and repair peptides

BPC-157 (Body Protection Compound)

Systemic healing peptide — gut, tendon, ligament, nerve repair

VERIFY FDA STATUS

COMPOUNDED

DOCUMENTED BENEFITS

- Accelerated tendon and ligament healing — strong animal data
- Gut mucosal healing — inflammatory bowel, leaky gut support
- Nerve regeneration support
- Anti-inflammatory — systemic and localized
- Routes: oral capsule (gut-focused), sublingual, or subcutaneous injection

IMPORTANT REGULATORY NOTE

- FDA reclassified BPC-157 as a bulk drug substance — compounding status has been restricted
- Verify current legal compounding status before prescribing
- Some pharmacies continue to compound — ensure pharmacy is aware of and compliant with current guidance
- Human clinical trial data is limited — most evidence from animal models

- Do not make efficacy claims beyond what evidence supports

Skin, hair, and aesthetic peptides

GHK-Cu (Copper Peptide)

Collagen stimulation, wound healing, hair follicle support

TOPICAL OTC

INJECTABLE RX

CLINICAL APPLICATIONS

- Collagen and elastin stimulation — well-documented
- Skin remodeling and wound healing — strong human data
- Hair follicle stimulation — emerging evidence for androgenic alopecia
- Anti-inflammatory and antioxidant properties
- Available as topical serum (cosmetic) or compounded injectable/subcutaneous
- Best aesthetic application: post-procedure healing acceleration

HOW TO INCORPORATE IN AESTHETICS

- Topical GHK-Cu serum: recommend post-microneedling, post-peel for enhanced healing
- Pairs perfectly with existing aesthetic treatments — natural extension of your current offering
- Compounded injectable for systemic skin benefits — prescribing provider required
- Hair loss protocol: topical + PRP + microneedling to scalp — comprehensive program
- Retail opportunity: high-quality topical GHK-Cu serum for home use

Sexual health peptides

PT-141 (Bremelanotide)

FDA-approved melanocortin receptor agonist for female sexual dysfunction

FDA APPROVED – VYLEESI

PRESCRIPTION

CLINICAL USE

- FDA-approved (Vyleesi) for hypoactive sexual desire disorder (HSDD) in premenopausal women
- Dose: 1.75mg subcutaneous injection 45 minutes before anticipated sexual activity
- Mechanism: acts centrally on melanocortin receptors — not vascular like PDE5 inhibitors
- Also prescribed off-label for men with erectile dysfunction and low desire
- Available commercially and through compounding pharmacies

PATIENT SELECTION AND COUNSELING

- Ideal for patients where low libido persists despite hormone optimization
- Common side effects: nausea (most common), flushing, headache — transient
- Avoid in patients with cardiovascular disease or uncontrolled hypertension
- Monitor blood pressure — can transiently decrease BP then increase
- Pairs naturally with testosterone optimization for women with HSDD
- High satisfaction rates when patient is properly selected and counseled

Sleep and cognitive peptides

Epitalon (Epithalamin)

Pineal peptide — sleep quality, telomere support, anti-aging

COMPOUNDED

VERIFY STATUS

PROPOSED MECHANISMS AND USES

- Tetrapeptide derived from pineal gland extract
- Proposed to regulate melatonin production and circadian rhythm
- Telomere lengthening claims — animal data only
- Typical protocol: 5–10mg daily subcutaneous for 10–20 day cycles
- Used for sleep optimization, longevity programs, anti-aging protocols

CLINICAL REALITY CHECK

- Human clinical evidence is limited — primarily Russian research
- Patient demand is growing — driven by longevity content online
- Be honest with patients about the evidence base
- Verify compounding availability and regulatory status before offering
- If offering, present as an emerging area with limited but promising early data

4

Vendors, Sourcing & Quality Control

Where to source peptides, how to vet pharmacies, and what to avoid

NEVER SOURCE PEPTIDES FROM RESEARCH CHEMICAL SUPPLIERS

A significant portion of peptides sold online are marketed as "research chemicals" and are not intended for human use. These products have no quality oversight, no purity testing, no sterility assurance, and no regulatory accountability. Prescribing or recommending these products to patients is both dangerous and potentially illegal. Only source from licensed compounding pharmacies with verifiable accreditation.

Compounding pharmacy partners for peptides

Empower Pharmacy

503B OUTSOURCING • FDA REGISTERED

One of the largest and most reputable 503B compounding outsourcing facilities in the US. FDA-registered. Sterile compounding capabilities. Broad peptide formulary — verify current peptide availability as formulary changes with regulatory updates. Competitive pricing. Ships nationwide. Prescriber account setup available.

→ Top choice for high-volume practices — institutional quality standards

Tailor Made Compounding

PCAB ACCREDITED • PEPTIDE SPECIALIST

Specialty compounding pharmacy with a strong peptide formulary. PCAB accredited. Known for quality and prescriber support. Offers GH secretagogues, PT-141, GHK-Cu, and others. Good prescriber education resources. Verify current formulary and regulatory compliance at time of ordering.

→ Strong choice for peptide-focused programs — good prescriber support

Carie Boyd's Prescription Shop / College Pharmacy

PCAB ACCREDITED

Long-standing accredited compounding pharmacies with peptide capabilities. College Pharmacy in Colorado Springs has a strong reputation in the functional medicine and peptide community. Verify current formulary. Good option for practices in states where larger outsourcing facilities don't ship.

→ Established reputation in functional medicine community

Defy Medical

TELEHEALTH + COMPOUNDING PLATFORM

Telehealth and compounding platform specializing in hormone optimization and peptide therapy. Option for collaborative arrangements — they prescribe, you refer. Good for practices not yet ready to prescribe peptides independently. Also a model to learn from — study their patient education approach.

→ Referral option if not prescribing peptides yet

Vetting your compounding pharmacy — the checklist

BEFORE YOU PRESCRIBE A SINGLE PEPTIDE — VERIFY ALL OF THE FOLLOWING

- PCAB accreditation or 503B FDA registration — non-negotiable
- Certificate of analysis (COA) available for each batch — purity and potency verified by independent third-party testing
- Sterility testing documentation for all injectable preparations
- Endotoxin (pyrogen) testing for injectables
- Current regulatory compliance — confirm each peptide you're prescribing is on their approved formulary and legally compoundable
- Cold chain shipping capability — peptides are temperature-sensitive
- Prescriber support line — you need to be able to reach a pharmacist with clinical questions

Patient sourcing — what to tell them

WHAT TO SAY WHEN A PATIENT ASKS ABOUT BUYING PEPTIDES ONLINE

"I understand the appeal — the internet is full of peptide sources that look legitimate. But the reality is that without a licensed compounding pharmacy, there's no way to verify what's actually in the vial. Purity, potency, sterility — none of that is guaranteed. The risk is real and it's not worth it. When we prescribe through an accredited pharmacy, you know exactly what you're getting and you have a

provider supervising your care. That's not just a legal formality — it's the difference between a program that works safely and one that may not work at all."

5

Pre & Post Consent Forms — Peptide Therapy

Informed consent for peptide programs — what must be documented

ATTORNEY REVIEW REQUIRED BEFORE CLINICAL USE

Peptide therapy carries unique regulatory complexity. Consent forms for compounded off-label peptides require careful legal review. Have a healthcare attorney in your state review all consent documents before using with patients. This template covers required clinical elements — not legal advice.

PRE-TREATMENT Informed Consent — Peptide Therapy

Patient reads, initials each section, signs at bottom. Provider co-signs. Keep in patient chart.

1. Nature of treatment: I understand I am being offered peptide therapy — a treatment involving short-chain amino acid signaling molecules — as part of a supervised wellness or medical program. My provider has explained the specific peptide(s) to be used and their intended purpose. _____initials

2. Off-label and compounded status: I understand that the peptide(s) prescribed to me may be: (a) compounded by a licensed compounding pharmacy and not FDA-approved as a finished drug product, and/or (b) prescribed for uses not specifically FDA-approved (off-label use). The use of compounded and off-label therapies is a legal and common medical practice. I accept this and consent to treatment. _____initials

3. Evidence base: I understand that some peptides have robust clinical evidence supporting their use, while others have more limited human data and are used based on emerging research, mechanism of action, and clinical experience. My provider has discussed the evidence level for the specific peptide(s) I am receiving. _____initials

4. Potential benefits: Potential benefits may include improvements in body composition, energy, recovery, sleep quality, skin and hair quality, sexual function, and/or cognitive function depending on the peptide(s) prescribed. I understand results vary and are not guaranteed. _____initials

5. Known risks and side effects: I have been informed of the following potential side effects specific to my prescribed peptide(s): injection site reactions (redness, swelling, bruising), water retention, fatigue, flushing, nausea, headache, tingling or numbness, changes in blood glucose, and changes in appetite. Additional peptide-specific risks have been discussed and are documented in my chart. _____initials

6. Cancer precaution: I understand that growth-promoting peptides (GH secretagogues) should not be used in the presence of active malignancy, as growth-promoting effects may theoretically stimulate tumor

growth. I confirm I have no known active malignancy. I agree to notify my provider immediately if I receive a cancer diagnosis while on therapy. _____initials

7. Injection training: If self-administering injections, I confirm that I have received training on proper subcutaneous injection technique, sharps disposal, storage requirements, and signs of injection site infection. I understand when to contact my provider with concerns. _____initials

8. Monitoring and follow-up: I agree to attend follow-up appointments and complete requested lab work as scheduled. I understand that ongoing monitoring is required for safe peptide therapy and that my prescription may be discontinued if I do not comply with monitoring requirements. _____initials

9. Regulatory environment: I understand that the regulatory status of compounded peptides is subject to change by the FDA, and that my provider may need to adjust, substitute, or discontinue my prescription based on changes in regulatory guidance. _____initials

10. Pregnancy: I confirm I am not pregnant or planning pregnancy during the course of this therapy. I understand peptide therapy is generally contraindicated in pregnancy. I will notify my provider immediately if I become pregnant. _____initials

Patient signature: _____ **Date:** _____

Provider signature: _____ **Date:** _____

Witness: _____ **Date:** _____

POST-INITIATION Check-In — 4-Week Follow-Up

Completed at the 4-week follow-up — in person or via telehealth

Symptom response: I have discussed my response to peptide therapy with my provider. My primary goal symptom(s) have: ☐ Improved ☐ Partially improved ☐ No change yet ☐ Worsened. Notes:

Side effects experienced: Side effects since starting therapy (write "none" if none): _____. My provider has reviewed these with me. _____initials

Injection technique: ☐ Comfortable with self-injection ☐ Requires additional instruction ☐ Provider-administered only

Protocol adjustment: My protocol has been: ☐ Continued as prescribed ☐ Dose adjusted ☐ Frequency

adjusted ☐ Peptide changed. Reason: _____

Continued consent: I have reviewed my progress with my provider and consent to continue peptide therapy as currently prescribed. _____ initials

Patient signature: _____ **Date:** _____

Provider signature: _____ **Date:** _____

6

Do's & Don'ts — Peptide Therapy in Practice

The clinical, operational, and legal rules that protect your patients and your practice

DO — ALWAYS

- ✓ Verify the current FDA compounding status of every peptide before prescribing
- ✓ Use only PCAB-accredited or 503B FDA-registered compounding pharmacies
- ✓ Request and review COA (certificate of analysis) for each batch
- ✓ Obtain written informed consent before initiating any peptide program
- ✓ Document the clinical indication for every prescription clearly
- ✓ Obtain baseline labs appropriate for the peptide category being prescribed
- ✓ Schedule 4-week follow-up and monitor for side effects and response
- ✓ Train patients on proper injection technique if self-administering
- ✓ Teach patients proper sharps disposal — OSHA compliance
- ✓ Be honest with patients about evidence levels — emerging vs. established
- ✓ Stay current on regulatory updates — FDA reclassifications happen without warning
- ✓ Have consent forms reviewed by a healthcare attorney in your state

DON'T — NEVER

- ✗ Source peptides from research chemical suppliers or unverified online sources
- ✗ Prescribe or recommend peptides from sources without verified pharmacy accreditation
- ✗ Make efficacy claims beyond what the evidence supports
- ✗ Prescribe GH secretagogues to patients with active malignancy
- ✗ Skip baseline labs to "save the patient money" — this is clinical and legal risk
- ✗ Prescribe without a follow-up plan — abandonment liability
- ✗ Continue prescribing a peptide whose compounding status has changed without verifying legality
- ✗ Allow patients to self-source peptides and then "supervise" the program
- ✗ Market peptide therapy as anti-aging cures or guaranteed fat loss
- ✗ Prescribe outside your scope of practice or without required prescribing authority
- ✗ Ignore patient-reported side effects — document every concern and your response

✗ Stack multiple peptides without understanding interaction profiles

Building the peptide program into your practice — step by step

- 1 Start with one peptide category — not the entire formulary**
Choose one area: body composition (GH secretagogues) OR healing (BPC-157 if available) OR skin (GHK-Cu). Master one category first. Build your protocols, consent forms, and patient education. Expand from a position of confidence, not overwhelm.
- 2 Establish your pharmacy relationship first**
Before you see your first peptide patient, have your compounding pharmacy relationship established, your prescriber account set up, and your formulary confirmed. Nothing is more professionally embarrassing than promising a patient a protocol you can't yet deliver.
- 3 Add peptide screening to your existing intake forms**
"Are you interested in peptide therapy for body composition, recovery, or anti-aging?" — one question surfaces interested patients from your existing base. You likely have patients right now who would sign up immediately.
- 4 Price it as a program, not a product**
Monthly program fee (\$200–\$400) covers provider management. Peptide cost billed separately through pharmacy. Labs billed separately. This creates three revenue streams from one patient relationship and makes the ongoing monitoring feel included rather than additional.
- 5 Cross-sell deliberately — not aggressively**
"Your GH secretagogue program is going to improve your body composition and skin quality over the next 3–6 months. A lot of patients doing this program love combining it with microneedling or filler — the skin responds better when the foundation is optimized." That's clinical, not sales.

7

Patient FAQs — Peptide Therapy

The questions every peptide patient asks — answered clearly and compliantly

What exactly are peptides and how are they different from hormones?



Are peptides safe?



Do I have to inject peptides? Is there another option?



How long until I see results?



Can I take peptides if I'm already on hormone therapy?



Why can't I just buy peptides online? They're cheaper.



Will peptides help me lose weight?



What does a peptide program cost?



NO PRIOR AUTHORIZATION · DANIELLE ALCALA · NOPRIORAUTHORIZATION.COM · FOR LICENSED PRESCRIBING PROVIDERS ONLY